

Surgery for eye cancer (ocular melanoma)

Surgery for eye cancer may involve removing part of the eye or sometimes the whole eye.

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About surgery for eye cancer

If you need eye surgery for [eye cancer](#), the type of operation will depend on things such as:

- the type of eye melanoma
- its size
- its position in your eye
- how it is affecting your sight
- your general health
- your preferences.

The thought of having any type of surgery to your eye can be worrying. Your specialist eye surgeon (ocular oncologist) and nurse will talk to you about the type of operation you need. They will explain what to expect. They will tell you whether the operation is likely to affect your eyesight (vision) or how you look. They will also give you advice on how to prepare for your operation. Find out more about [preparing for surgery](#).

There are different operations for eye melanoma.

Removing part of the eye (local resection)

Surgery is the most common treatment for [conjunctival melanoma](#). The surgeon removes the tumour from the surface of the eye, along with a small amount of surrounding healthy conjunctiva. They may also use a treatment called cryotherapy. This destroys any remaining cancer cells by freezing them.

You do not usually have surgery for [uveal melanoma](#). Sometimes a surgeon may remove the tumour through a cut (incision) they make in the eyeball. They also remove a small amount of healthy tissue around the tumour. You have this operation under general anaesthetic.

After surgery, you have eye drops to reduce the risk of infection and allow the wound to heal.

You usually have [radiotherapy](#) after surgery for conjunctival melanoma. It is given to reduce the risk of melanoma coming back.

If you have surgery for uveal melanoma you usually have radiotherapy using a small radioactive disc put on the outside of the eye after it. This is called [brachytherapy](#).

Removing the whole eyeball (enucleation)

Removing the whole eyeball is called enucleation. Doctors will only suggest this if they are certain it is the best treatment for you. This may be if:

- the tumour is too large to be treated by [radiotherapy](#).
- radiotherapy to the area could have serious side effects
- the eye becomes painful after radiotherapy and your vision is also badly affected.

You will need time to discuss this operation with your cancer doctor and nurse before you make any decision.

Your doctor and nurse can usually arrange extra support to help you cope. This may include counsellors or patient support groups. They may be able to put you in touch with someone who has had the same operation.

The operation

During the operation, the surgeon removes the eyeball and puts an eye-shaped implant into your eye socket. They cover the implant with healthy surrounding tissue. The implant fills the space where your eyeball was. The surgeon attaches your eye muscles over the implant. They put in a plastic shell to keep the shape of the eyelid. This is called a conformer. The socket is then padded.

You may have this operation under general anaesthetic. Or you may have it under local anaesthetic. The drugs will make you feel drowsy and help relax you.

After the operation

You usually go home 1 to 2 days after the operation. The nurses will explain how to look after your wound. The missing eye will be covered with a pad and dressing. This is kept in place for 3 to 5 days to reduce swelling in the eye socket. The eye socket is then left unpadded so it can heal.

Your eyelid may be swollen for a short period of time. Some people find it helpful to wear dark glasses until the swelling goes down.

Several weeks after the operation, you have an artificial eye made. This is called a prosthesis. A specialist who makes and fits artificial eyes is called an ocular prosthetist. They will match the shape, colour and size of your other eye as closely as possible. Your prosthesis should look natural and have

some movement. You wear the prosthesis permanently. You only have to remove it occasionally to clean it.

Effects on your eyesight

Having one eye removed will affect your perception of depth and reduce your field of vision. These are things you can usually adapt to.

You can still drive afterwards, but not until a few weeks after surgery. This allows time for your vision to adapt. There may be restrictions on the types of vehicles you are allowed to drive. You must tell the [DVLA](#) or [DVA in Northern Ireland](#) after having one eye removed. You must meet their vision standard before starting to drive again.

Removing the eye and surrounding tissue (orbital exenteration)

This type of surgery is rarely needed. It may be used to treat a large [conjunctival melanoma](#) that has spread to areas around the eye. The surgeon removes the eyeball, along with the socket structures and the eyelid.

It is a big operation that affects your appearance. You will need a lot of support from your healthcare team before surgery. You may meet a counsellor or psychologist for specialised support.

After the operation, you may have a facial prosthesis made. This is an artificial part of your face that covers the eye socket with false eyelids, lashes and an artificial eye. The false eye will not be able to move or blink. You can wear a facial prosthesis on a pair of glasses. Or you may have it fixed to your face with special glue. Sometimes the prosthesis can be fitted to studs the surgeon fixes into the bone around the eye socket.

About our information

References



Below is a sample of the sources used in our eye cancer (ocular melanoma) information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Jain P, Finger PT, Fili M, et al. Conjunctival melanoma treatment outcomes in 288 patients: a multicentre international data-sharing study. *British Journal of Ophthalmology* 2021;105:1358–1364. (accessed May 2022).

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Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, Professor Samra Turajlic, Consultant Medical Oncologist.

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Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

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⚠ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help